جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : نيللى عبداللطيف عبداللطيف السيد
المحافظة : الجيزه **الحي / المدينة :** الهرم **العنوان :** فيصل
رقم البطاقة : 27709190104126 **رقم التليفون :** 01275221878 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 19-09-1977 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نيالى عبداللطيف عبداللطيف السيد
National ID: 27709190104126

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID • Front's Back
[[DOCTYPE.ID_FRONT_BACK]]

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Patient Name	: الأستاذه / نيللي عبد اللطيف عبد اللطيف	Request No.	: 13005263
Patient No.	: 27709190104126	Request Date	: 02/03/2026
Path No.	: 582/26	Verification Date	: 08/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 48 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast mass, (BIRADs 4c).
Nature of Specimen: Tru-cut biopsy, collected on 2/3/2026, at 9:35 am, Cold ischemia time: 20 seconds.

Gross Examination:
- Fragmented grey white tissue cores, ranging in size between 0.3 cm & 1.5 cm long, totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
* Ductal carcinoma in situ: Absent.
* Perineural invasion: Absent.
* Angioinvasion: Absent.
* Necrosis: Focal.

Diagnosis:
Left breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 3.
- For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Walaa Ibrahem Ghonaim.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: نيللى عبداللطيف عبداللطيف السيد **MR. :** 9935	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient has a history of hypertension, is married with four offspring, and was diagnosed with invasive ductal carcinoma (IDC) of the left breast measuring 5 × 3.3 cm with involvement of more than three axillary lymph nodes; the tumor is triple-negative on staging work-up.

Multidisciplinary discussion in June 2026 concluded that, given cT3 N1+ M0 triple-negative breast cancer (TNBC), the treatment plan should include port-a-cath insertion, clip placement, neoadjuvant chemotherapy (NACT) with weekly Taxol and carboplatin (planned for 12 cycles) followed by four cycles of AC, concomitant pembrolizumab (Keytruda) per the KEYNOTE-522 protocol, BRCA testing, and consideration of left breast wide local excision with axillary lymph-node management based on response.

On 9 August 2026 the patient received week 1 (W1) of Taxol 75 mg/m² and carboplatin AUC 2, with planned continuation of weekly treatment; pembrolizumab 200 mg was scheduled to start after laboratory results (height 153 cm, weight 62 kg). The same regimen was ordered for weeks 2 (W2) and 3 (W3) through the pharmacy.

On 16 August 2026 the clinical pharmacy note recorded the patient's weight as 62 kg, height 158 cm (body surface area 1.62 m²), and confirmed dosing for week 2 (W2) Taxol 122 mg and carboplatin AUC 2.

On 23 August 2026 the pharmacy documented continuation of the regimen with week 3 (W3) Taxol and carboplatin AUC 2, along with the first dose of Keytruda (C1).

On 30 August 2026 the medical oncology note described the left breast cancer as cT3 N1 M0 IDC, triple-negative, Ki-67 80 %, estrogen receptor 67 %. Imaging showed a right hepatic cyst, a few tiny pulmonary nodules, and a right breast BI-RADS 3 lesion. The patient received week 4 (W4) Taxol 122 mg, carboplatin AUC 2, and one dose of pegfilgrastim (Neupogen) in preparation for clip insertion.

On 6 September 2026 the internal medicine note reiterated the multidisciplinary plan (port-a-cath, clip insertion, NACT Taxol/Carboplatin ×12 followed by AC ×4 with concurrent Keytruda per KEYNOTE-522, BRCA testing, and surgical considerations). It also referenced upcoming treatment weeks: W5 with taxane-carboplatin, W6 with taxane-carboplatin plus Keytruda, and W7 in preparation for surgery.

The patient has completed four weeks (W4) of the taxane-carboplatin neoadjuvant regimen to date.

تحريرا في ,

التاريخ :	19-09-2026	الوقت :	

الطبيب المعالج : نهي سعيد علي محمد

يعتمد ,

GUSTAVE ROUSSY
INTERNATIONAL CANCER CAMPUS

Patient Name	: الأستاذه / نيللي عبد اللطيف عبد اللطيف	Request No.	: 13005263
Patient No.	: 27709190104126	Request Date	: 02/03/2026
Path No.	: 582/26	Verification Date	: 08/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 48 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass, (BIRADs 4c) diagnosed "Invasive duct carcinoma grade 3".
Nature of Specimen: Tru-cut biopsy, collected on 2/3/2026, at 9:35 am, Cold ischemia time: 20 seconds.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8)
- PR: Negative nuclear reaction in tumor cells (score 0/8)
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (Ultra-low, score 0+).
- Ki67%: 80%.

Conclusion:
Left breast mass, Tru-cut biopsy:
IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Ultra low (score 0+).
- Ki67%: 80%.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / نيللي عبد اللطيف عبد اللطيف	Request No.	13005263
Patient No.	27709190104126	Request Date	02/03/2026
Path No.	582/26	Verification Date	08/03/2026
Sex	Female	Financial Category	جوستاف روسي - مبادره
Age	48 Y	Department	Out Patient
Referreing Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0 (Ultra-low): as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghonaim

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

اسم المريض بالإنجليزية: nilli Abdallatif Abdallatif	كود المريض: 93582
تاريخ التقرير: 2026/09/01 01:04:29 PM	السن: ٤٨ سنة, ١١ شهر

Ultrasound Guided clip insertion:

Clinical summary: Pathologically proven Left breast carcinoma for clip insertion.

Technique:

- Under complete sterile conditions one metallic clip is inserted within the center of large malignant mass lesion at the left breast and measures today about 4 x 2 cm and located at 2 o'clock position.
- This was confirmed by mammography CC & ML views.

Best Regards
Dr. Iman Adel
Breast Imaging consultant
& Breast Intervention

جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

07/01/2026

MDT FORM

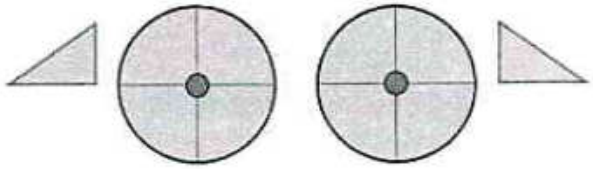
Name:	نيللي عبداللطيف عبداللطيف السيد	**MRN:**	9935
Age:	49 Y	**ID:**	27709190104126
Menopausal Sta	pre	**Phone:**	01275221878
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:	HTN	**Reason Of MDT:**	new

Clinical Data:

BX: IDC G 3

ER: 0/8 **PR:** 0/8 **HER2:** 0 **KI67:** 80%

R L



TNM

Sonomammography:

UOQ at 1-2 o'clock oval shaped indistinct heterogenous hypoechoic vascular palpable lesion measuring about 5X3.3 cm (8 cm from the nipple, abutting overlying skin) ? Normal skin and nipple/areolar complex.

• More than three axillary level I LNs showing non-uniform cortical thickening reaching 0.39 cm and effaced hilum.

Impression:

? Left breast (BIRADS 4c): UOQ highly suspicious lesion for US guided biopsy. ? Right breast (BIRADS 3) UOQ benign looking lesion for follow up.

Mets Workup:

CT:

Few tiny right pulmonary nodules. • Right hepatic lobe small lesion likely cyst. • Bulky left suprarenal gland. • Posterior uterine fibroid.

bone scan : free

ECHO normal

History:

Recommendations:

MDT Outcome & Plan

In view of having cT3 N1+ M0, TNBC, for:
1.Portacath insertion
2.Clip insertion
3.Start NACT Taxol/Carbo x12 followed by AC x4 with concomitant \keytruda "Keynote 522"
4.BRCA testing
5.Consider Lt WLE & axillary LN management according to response.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

1/7/2026 17/7/2026

جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

07/01/2026

MDT FORM

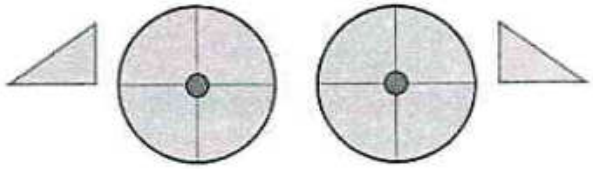
Name:	نيللي عبداللطيف عبداللطيف السيد	**MRN:**	9935
Age:	49 Y	**ID:**	27709190104126
Menopausal Sta	pre	**Phone:**	01275221878
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:	HTN	**Reason Of MDT:**	new

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BX: IDC G 3

ER: 0/8 **PR:** 0/8 **HER2:** 0 **KI67:** 80%

R L



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5.Consider Lt WLE & axillary LN management according to response.

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MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

1/7/2026 17/7/2026


جمهورية مصر العربية
بطاقة تحقيق الشخصية
نيللى
عبداللطيف عبداللطيف السيد
١٣ش الصفاوالمروه-مدينة الرحمن-فيصل
الهرم - الجيزه
٢٧٧ ٠٩ ١٩ ٠١ ٠٤١ ٢٦
LT5732449


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٢٠٢٦/٠٥
بدون عمل
انثى مسلمة متزوجة
صلاح مصطفى عبدالمقصود احمد
البطاقة سارية حتى ٢٠٣٣/٠٥/١٨

جمهورية مصر العربية
بطاقة تحقيق الشخصية



نبيلى
عبداللطيف عبداللطيف السيد
ش الصفاوالمروه-مدينة الرحمن-فيصل
الهرم - الجيزه

٢٧٧٠٩١٩٠١٠٤١٢٦

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بدون عمل
٢٠١٩/٠٥

١
انثى مسلمة
صلاح مصطفى عبدالمقصود احمد متزوجة
البطاقة سارية حتى ٢٠٢٦/٠٥/٢١



١٢٧٥٢٢١٨٧٨
١٢٧٤٥٥٢٣٩٠

Patient ID: 93581	Patient Name English: nilli Abdallatif Abdallatif
Age: ٢٨ سنة, ٥ شهر	Report Date: 2026/02/25 11:05:26 AM

Women's Health Outreach Program

Bilateral Full field Digital Mammography

Clinical summary: Left breast lump.

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Left breast

- Scattered fibro glandular densities (ACR b).
- UOQ posterior 1/3 partially circumscribed partially indistinct oval shaped hyperdense lesion.
- **Ultrasound** demonstrated UOQ at 1-2 o'clock oval shaped indistinct heterogenous hypoechoic vascular palpable lesion measuring about 5X3.3 cm (8 cm from the nipple, abutting overlying skin).
- Normal skin and nipple/areolar complex.
- More than three axillary level I LNs showing non-uniform cortical thickening reaching 0.39 cm and effaced hilum.

Right breast

- Scattered fibro glandular densities (ACR b).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated UOQ at 11 o'clock circumscribed oval shaped iso to hypoechoic small lesion measuring 0.9X0.4 cm.
- Few small cysts.
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Impression:

- **Left breast (BIRADS 4c): UOQ highly suspicious lesion for US guided biopsy.**
- **Right breast (BIRADS 3) UOQ benign looking lesion for follow up.**

Dr. Amira Shaheen, MD

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / نيللي عبد اللطيف عبد اللطيف	Request No.	: 13005263
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Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 48 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast mass, (BIRADs 4c).
Nature of Specimen: Tru-cut biopsy, collected on 2/3/2026, at 9:35 am, Cold ischemia time: 20 seconds.

Gross Examination:
- Fragmented grey white tissue cores, ranging in size between 0.3 cm & 1.5 cm long, totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
* Ductal carcinoma in situ: Absent.
* Perineural invasion: Absent.
* Angioinvasion: Absent.
* Necrosis: Focal.

Diagnosis:
Left breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 3.
- For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Walaa Ibrahem Ghonaim.

Verified by :
Dr.Duaa Mohamed

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غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
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GUSTAVE ROUSSY
INTERNATIONAL

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Patient No.	: 27709190104126	Request Date	: 02/03/2026
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Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 48 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

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Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8)
- PR: Negative nuclear reaction in tumor cells (score 0/8)
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (Ultra-low, score 0+).
- Ki67%: 80%.

Conclusion:
Left breast mass, Tru-cut biopsy:
IPT:
- ER: Negative (score 0/8).
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- Her2: Ultra low (score 0+).
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ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

Verified by :
Dr.Duaa Mohamed

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name : Nelly Abdul Latif Abdul Latif El-Sayed	Date : 20-06-2026 08:30
Branch : Maadi	Medical No. : 397774
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : تعاقد مستشفيات

Whole-Body Bone Scintigraphy Report

Clinical History

- A 48-year-old female presenting with history of left breast carcinoma.
- The current whole-body bone scintigraphy is performed for initial staging and evaluation of potential skeletal metastases.

Technical Procedure: Following the intravenous administration of Tc-99m MDP, routine static whole-body anterior and posterior planar images were acquired approximately 3 hours post-injection.

Scintigraphic Findings

- **Skeleton:** Normal, symmetrical radiotracer distribution throughout the entire axial and appendicular skeleton. No focal areas of abnormal increased or decreased osteoblastic activity identified within the skull, spine, ribs, sternum, pelvis, or extremities.
- **Soft Tissues:** Normal physiological renal excretion and urinary bladder activity. No abnormal extraosseous tracer accumulation.

Conclusion

- **Normal Whole-Body Bone Scan.** No scintigraphic evidence of active osteoblastic skeletal metastases at the present time.
- **Recommendation:** Follow-up imaging as per oncological protocol.

Dr. Alaa Nagi, MD
Nuclear Medicine

6

NILE DIAGNOSTIC CENTER

GUSTAVE ROUSSY INTERNATIONAL HOSPITAL

جوستاف روسي انترناشونال مصر

ECHO REPORT

NAME : نللي عبد اللطيف عبد اللطيف

DATE: 8 / 6 /2026

ID NUMBER : /

SEX : AGE:

EF	67%	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.7	3.5-5.8cm	AO.R	2.7	3-3.7cm
LVESD		2-4cm	LA	3.2	2-4cm
FS		25-45%	RV		0.7-2.3com(supine)
SWT	09	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions ——— oimd

LV Wall Thickness ——— oimd

LV Diastolic Function ——— oimd

SWMA ——— SWMA

LA and Ao Root Dimensions ——— oimd

Mitral Valve ——— oimd

Aortic Valve ——— oimd

Tricuspid Valve Trivial TR

Pulmonary Valve ——— oimd

Right Side and Systolic Function ——— oimd

Intracardiac Masses and Effusion ———

DR.

د/ميادة

Patient's Name:	**Nilly Abdellatif Abdellatif**
Date:	**2026-04-20**
Referred by:	Prof.Dr. **CAPwithC**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

A known case of left breast cancer for metastatic workup

I- CHEST:

- The left breast showed UOQ soft tissue mass lesion with central necrosis measures 5.1x4.9x5.7 cm in its maximum AP,TS and CC dimensions respectively, the lesion is seen resting/ inseparable from underlying pectoralis muscle. Associated with multiple enlarged left axillary lymph nodes, the largest measures about 19.9x19.6 mm **for sono-mammography.**
- Prominent right axillary lymph nodes, the largest measures about 9.6mm
- Few tiny right pulmonary nodules, the largest is seen in right upper anterior segment measures about 3.5mm
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. Right hepatic lobe segment VIII showed tiny non enhanced focal lesion measures about 5 mm
- No dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen (splenules, the largest measures about 1.3 cm) , pancreas, right suprarenal gland, aorta and IVC. Bulky left suprarenal gland.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.

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- Bulky uterus with posterior uterine wall fibroid measures about 2x1.8 cm
- left ovarian cyst measures about 1.5 cm likely functional
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- **The examined skeleton** no worrisome lytic or sclerotic lesions.

Opinion:

- **Few tiny right pulmonary nodules.**
- **Right hepatic lobe small lesion likely cyst.**
- **Bulky left suprarenal gland.**
- **Posterior uterine fibroid.**

Dr.Nourhan Ramzy

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